Extremity Trauma

TB-04 | Trauma & Burns | Reviewed

Clinical release pending. Verify current approved protocols and standing orders.

Quick flow

01 Extremity Injury [EMT / AEMT / Paramedic]

Expose, identify life-threatening hemorrhage, and assess associated trauma.

02 Major Hemorrhage? [EMT / AEMT / Paramedic]

Direct pressure • approved wound packing • tourniquet for life-threatening extremity bleeding when pressure is ineffective or impractical. Tighten until bleeding stops and distal pulse is absent; reassess after movement.

03 Check CSM [EMT / AEMT / Paramedic]

Document distal pulse, capillary refill, skin color/temperature, sensation, and movement before and after every alignment, splint, or tourniquet.

04 Align / Splint [EMT / AEMT / Paramedic]

Cover open wounds, stabilize joints above and below, and reassess CSM. Gently realign only for absent distal perfusion or when needed to splint; stop for resistance or worsening pain. Do not force reduction.

05 Pain / Shock Care [AEMT / Paramedic]

Prevent hypothermia and provide authorized analgesia and vascular access.

06 Appropriate Destination [EMT / AEMT / Paramedic]

Apply the trauma field-triage and destination plan: high-risk patients to the highest-level appropriate trauma center within regional constraints; favor pediatric-capable trauma care for children. Use a closer stabilization hospital only for an immediate critical need that cannot safely be managed during longer transport, under the destination plan/Medical Control. Notify early, document the reason, and activate the predefined transfer process. Do not delay ground movement solely while awaiting aircraft.

Overview

- Prioritize hemorrhage control and neurovascular preservation in extremity injury.
- Treat amputations, open fractures, dislocations, crush injury, and compartment syndrome as time-sensitive trauma.

Provider-Level Actions

Perform all immediately indicated care within scope without delaying rapid trauma transport.

EMT

- For life-threatening extremity hemorrhage, apply an approved tourniquet promptly when direct pressure is ineffective or impractical. Tighten until bleeding stops and the distal pulse is absent. If bleeding persists despite proper placement and tightening, add a second tourniquet immediately proximal to the first when feasible. Record time, leave visible, and reassess after movement and throughout transport. For suitable junctional wounds, pack with approved gauze and maintain direct pressure per product instructions. Document distal pulse, capillary refill, skin color/temperature, sensation, and movement before and after every intervention.
- For open fractures, cover with a sterile dressing; do not probe the wound or push exposed bone back in. Wrap amputated tissue in moist sterile gauze, seal it in a labeled bag with patient/time, send it with the patient, and keep cool without direct contact with ice or water.

AEMT

- Perform all EMT care plus vascular access, fluid therapy for shock, and authorized analgesia.

- Expanded AEMT analgesia is not enabled by default while individual authorization is unverified. The specified medication, route, and patient group require current Tennessee authority, documented Tennessee-approved expanded analgesia training, agency credentialing, and medical-director standing orders; training alone does not grant permission.
- Trend distal perfusion and report any deterioration immediately.

Paramedic

- Perform all prior care plus advanced pain management and resuscitation of hemorrhagic shock within protocol.
- Coordinate direct trauma-center transport when limb or life is threatened.

Indications

- Patient with suspected extremity trauma requiring prehospital assessment or treatment.

Warnings / precautions

- Do not delay transport attempting perfect anatomic alignment. Do not force fracture or dislocation reduction; attempt gentle realignment only for absent distal perfusion or when needed to splint, and stop for resistance or worsening pain.
- Record tourniquet application time. Do not periodically loosen or remove a tourniquet. A pulseless extremity, expanding hematoma, severe pain out of proportion, or tense swelling requires urgent specialty care.

Source and references

Claiborne County EMS native protocol content; controlled PDF generated from the same structured source. | Current Tennessee EMS scope of practice, verified individual credentials, and Claiborne County standing orders control provider authorization.

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